Stye & Chalazion Self-Assessment Guide

Questions and Detailed Answers

SECTION 1: Core Symptoms

Question 1: Do you have a bump or lump on your eyelid?

Answer Options:

- [ ] Yes
- [ ] No

Detailed Explanation:

If you answer **NO**, you likely don't have a stye or chalazion. These conditions are characterized by visible or palpable lumps on the eyelid. The eyelid may appear normal from the outside in very early stages, but typically there is swelling or a noticeable bump.

If you answer **YES**, proceed with the remaining questions to determine the type, severity, and appropriate action.

Question 2: Where is the bump located?

Answer Options:

- [ ] On the edge of the eyelid, near the eyelashes
- [ ] Inside the eyelid (you can feel it when you close your eye or flip the lid)
- [ ] Away from the eyelid edge, deeper in the lid
- [ ] Not sure

Detailed Interpretation:

On the edge, near eyelashes → Likely **external stye (hordeolum)**

- Infection of eyelash follicle or Zeis/Moll glands at lid margin
- Appears as red, painful bump pointing outward
- May see small yellow pus head at the tip
- Usually more painful than chalazion
- Typically resolves in 5-7 days with warm compresses

Inside the eyelid → Likely **internal stye or chalazion**

- Internal stye: Infection of meibomian gland, acute and painful
- Chalazion: Blocked meibomian gland, chronic and usually painless
- You may need to flip the eyelid to see redness or swelling on inner surface
- Can cause more eyelid swelling than external stye
- May point toward conjunctival side

Away from edge, deeper → Likely **chalazion**

- Blocked oil gland (meibomian gland) deeper in the tarsal plate
- Firm, round, pea-sized lump
- Grows slowly over days to weeks
- Usually painless unless inflamed
- Can persist for weeks to months without treatment

Not sure → Try gently palpating the eyelid with clean hands; if the lump is hard and painless, it's likely chalazion; if tender and red, likely stye

---

Question 3: Is the bump painful?

Answer Options:

- [ ] Yes, very painful and tender
- [ ] Yes, mildly uncomfortable
- [ ] No, painless
- [ ] Only hurts when I press on it

Detailed Interpretation:

Very painful and tender → Stye (hordeolum)

Pain is the hallmark of stye because it's an acute infection:

- Bacterial infection (usually *Staphylococcus aureus*) causes inflammation
- Infected gland fills with pus, creating pressure
- Nerve endings in eyelid are abundant, making it very sensitive
- Pain typically peaks in first 2-3 days
- May throb or ache, worsens with eye movement or touch
- **Management**: Warm compresses 4-6 times daily, pain relief (ibuprofen), see doctor if no improvement in 48 hours

Mildly uncomfortable → Inflamed chalazion or resolving stye

- Chalazion can become secondarily inflamed and cause mild discomfort
- Resolving stye may transition from pain to mild irritation
- Pressure sensation from swelling rather than infection
- Still requires warm compresses and monitoring

Painless → Chalazion

- Characteristic feature: painless, firm lump
- No active infection, just chronic blocked gland with granuloma formation
- May cause heaviness or awareness of lump
- Can be present for weeks to months
- If very large, may cause blurred vision by pressing on eyeball
- **Management**: Warm compresses, lid massage, if persistent >4-6 weeks consider steroid injection or surgery

Only hurts when pressed → Chalazion or encapsulated lesion

- Localized tenderness suggests contained inflammation

- Not actively infected
- May have fibrous capsule around blocked material
- Typically safe to observe with home care

Question 4: How long have you had this bump?

Answer Options:

- [ ] 1-3 days (just appeared)
- [ ] 4-7 days
- [ ] 1-3 weeks
- [ ] More than 1 month
- [ ] It comes and goes (recurrent)

Detailed Interpretation:

1-3 days (just appeared) → Acute stye

- Early stage of infection
- Rapid onset is typical of bacterial infection
- Redness, swelling, and pain develop quickly
- May still be developing, could worsen over next 24-48 hours
- **Action**: Start warm compresses immediately (4-6 times daily, 10-15 minutes each)
- Monitor closely; if worsening rapidly or spreading beyond localized area, see doctor

4-7 days → Peak period for stye OR early chalazion

- Stye typically peaks at 3-5 days, then should start improving
- If painful and red → likely stye at peak; should improve by day 7-10
- If painless and firm → may be early chalazion forming
- **Action**: Continue warm compresses; if stye not improving by day 7, see doctor for possible antibiotics

1-3 weeks → Chalazion OR persistent stye

- Most styes resolve by 2 weeks; persistence suggests:
  - Inadequate drainage
  - Large abscess requiring incision
  - Actually a chalazion, not stye
- Chalazion commonly becomes noticeable at 1-3 weeks as it slowly enlarges
- **Action**: See eye doctor for evaluation; may need steroid injection or incision/curettage

More than 1 month → Chronic chalazion (NEEDS DOCTOR)

- Chalazia can persist for months if untreated
- Unlikely to resolve spontaneously after 4-6 weeks
- May require surgical removal
- **Important**: Any eyelid lump persisting >1 month should be examined to rule out rare eyelid tumors (basal cell carcinoma, sebaceous cell carcinoma)
- **Action**: Schedule ophthalmology appointment; may need biopsy if atypical features

Recurrent (comes and goes) → Chronic blepharitis, meibomian gland dysfunction, or rosacea

- Recurring styes/chalazia suggest underlying eyelid inflammation
- May indicate:
  - Chronic blepharitis (lid margin inflammation)
  - Meibomian gland dysfunction (MGD)
  - Rosacea (skin condition causing oily skin, facial flushing)
  - Seborrheic dermatitis
  - Diabetes (impaired immune function)
- **Action**: See eye doctor for comprehensive evaluation; will need long-term eyelid hygiene regimen, possibly antibiotics or anti-inflammatory treatment

---

Question 5: How fast did the bump develop?

Answer Options:

- [ ] Appeared overnight or within 1 day
- [ ] Gradually grew over several days
- [ ] Slowly developed over 1-2 weeks
- [ ] Very slow growth over weeks to months

Detailed Interpretation:

Overnight or within 1 day → Acute stye

- Bacterial infections develop rapidly
- Wake up with red, swollen, painful eyelid
- Classic presentation of external or internal hordeolum
- Indicates active infection requiring prompt treatment

Gradually over several days → Stye or early inflamed chalazion

- Typical progression of stye
- May start as mild irritation, then become painful lump
- If pain is predominant → stye
- If firmness without much pain → chalazion

Slowly over 1-2 weeks → Chalazion

- Characteristic slow, painless growth
- Patient may not notice until it becomes cosmetically visible
- Indicates chronic blockage rather than acute infection

Very slow over weeks to months → Chalazion OR concerning lesion

- Large chalazia can grow very slowly
- **Warning**: Very slow-growing, irregular, or ulcerated lesions may rarely be eyelid cancers
- Features suggesting serious pathology:
  - Loss of eyelashes in area of lump (madarosis)
  - Hard, irregular texture
  - Fixation to underlying tissue

    - Ulceration or bleeding
    - Recurrence in exact same spot after previous treatment
- **Action**: Any atypical slow-growing eyelid lump requires ophthalmology evaluation and possible biopsy

---

SECTION 2: Associated Symptoms

Question 6: Is your eyelid red and swollen?

Answer Options:

- [ ] Yes, very red and swollen
- [ ] Yes, mildly swollen with localized redness
- [ ] No, just a firm lump with normal-looking skin
- [ ] Yes, and the redness is spreading to my cheek/eyebrow

Detailed Interpretation:

Very red and swollen → Active stye OR inflamed chalazion

- Significant inflammation present
- Entire eyelid may be puffy and red
- Indicates acute phase of infection or inflammation
- May have difficulty opening eye if severe
- **Management**: Warm compresses, may need oral antibiotics if cellulitis spreading

Mildly swollen with localized redness → Typical stye or inflamed chalazion

- Localized reaction around the lump
- Normal part of healing process
- Continue home care with warm compresses

No redness, just firm lump → Chronic chalazion

- Old, "cold" chalazion
- Minimal inflammation
- Fibrous capsule formed around blocked secretion
- Less likely to resolve with conservative treatment
- May require steroid injection or surgical removal

Spreading redness to cheek/eyebrow → PRESEPTAL CELLULITIS (EMERGENCY)

- Infection spreading beyond the eyelid
- Potentially serious, can progress to orbital cellulitis
- Requires systemic antibiotics
- **Warning signs**:
    - Fever
    - Increasing pain
    - Difficulty moving eye
    - Vision changes
    - Swelling of entire eye area
- **Action**: Seek emergency medical care immediately; may require hospitalization and IV antibiotics

---

Question 7: Do you have yellow pus or discharge?

Answer Options:

- [ ] Yes, yellow pus at the tip of the bump
- [ ] Yes, discharge crusting on my eyelashes
- [ ] No visible pus
- [ ] Yes, and it's draining or has already drained

Detailed Interpretation:

Yellow pus at tip → Stye ready to drain (GOOD SIGN)

- Abscess has come to a "head"
- Natural drainage will occur soon
- **DO NOT squeeze or pop it**
- Continue warm compresses to encourage spontaneous drainage
- Once drained, symptoms typically improve rapidly within 24-48 hours
- Keep area clean with gentle lid hygiene

Discharge crusting on eyelashes → Active infection

- Pus leaking from infected gland
- May glue eyelashes together, especially in morning
- Indicates bacterial infection (stye or infected chalazion)
- **Management**:
  - Warm compresses to soften crust
  - Gentle cleaning with diluted baby shampoo or lid wipes
  - May need antibiotic ointment
  - See doctor if not improving in 48 hours

No visible pus → Chalazion OR early stye

- Chalazion: No pus because it's not infected, just blocked
- Early stye: Infection not yet advanced to suppuration
- Continue observation and warm compresses

Draining or has drained → Resolution phase (EXCELLENT)

- Natural drainage is the goal of treatment
- Symptoms should rapidly improve after drainage
- Continue warm compresses for 2-3 more days to ensure complete drainage
- Keep area clean to prevent reinfection
- If symptoms don't improve after drainage, see doctor (may have deeper abscess)

Question 8: Is your vision affected?

Answer Options:

- [ ] Yes, blurry vision
- [ ] Yes, double vision
- [ ] No, vision is normal
- [ ] Only blurry when the swelling pushes on my eye

Detailed Interpretation:

Blurry vision → NEEDS DOCTOR

Simple stye or chalazion should NOT cause significant vision changes (except temporarily from tears or discharge coating the eye).

Persistent blurred vision suggests:

- **Large chalazion pressing on cornea**: Causes temporary irregular astigmatism
- **Corneal involvement**: Keratitis (corneal infection) from spread of infection
- **Intraocular inflammation**: Rare but serious complication
- **Orbital cellulitis**: Infection behind the eye, pressing on optic nerve

Action: Vision changes require ophthalmology examination within 24 hours

Double vision → EMERGENCY - SEE DOCTOR NOW

Double vision suggests involvement of eye muscles or orbital structures:

- **Orbital cellulitis**: Infection in eye socket causing muscle swelling and restricted movement
- **Abscess**: Collection of pus pressing on muscles
- **Cavernous sinus thrombosis**: Rare but life-threatening complication of facial infection

Action: Double vision with eyelid infection is an emergency; go to emergency department immediately

Vision is normal → GOOD SIGN

- Infection/inflammation limited to eyelid
- No corneal or intraocular involvement
- Safe to manage with appropriate treatment based on other symptoms

Blurry only when swelling pushes → Mechanical effect (MONITOR)

- Large chalazion or very swollen eyelid can temporarily distort corneal shape
- Vision clears when you open the eye wide or look away from the lump
- Not a serious concern but indicates large lesion
- Consider treatment to reduce size (warm compresses, doctor evaluation)

Question 9: Do you have a gritty or sandy feeling in your eye?

Answer Options:

- [ ] Yes, feels like something is in my eye
- [ ] No

Detailed Interpretation:

Yes, gritty sensation → Common and usually benign

Foreign body sensation occurs because:

- Swollen eyelid rubs against eyeball surface
- Inflammatory cells and debris on ocular surface
- Altered tear film from blocked meibomian glands
- Possible mild corneal irritation

Management:

- Preservative-free artificial tears 4-6 times daily
- Warm compresses
- Avoid rubbing eye
- If severe or worsening, see doctor to check for corneal abrasion or keratitis

No gritty feeling → Inflammation well-localized to eyelid

Question 10: Are you sensitive to light?

Answer Options:

- [ ] Yes, bright light hurts my eyes
- [ ] No

Detailed Interpretation:

Yes, photophobia → WARNING SIGN - NEEDS DOCTOR

Photophobia is NOT typical of simple stye or chalazion. Its presence suggests:

Corneal involvement:

- Keratitis (corneal infection)
- Corneal abrasion from rubbing
- Epithelial defect

Anterior uveitis (iritis):

- Inflammation inside the eye
- Associated with eye pain, blurred vision, ciliary flush (redness around iris)
- **Medical emergency** requiring immediate treatment

Other serious conditions:

- Acute glaucoma
- Endophthalmitis (rare)
- Severe conjunctival inflammation

Action: Photophobia with eyelid lump requires **same-day ophthalmology evaluation**

No light sensitivity → Good prognostic sign

SECTION 3: Medical History & Risk Factors

Question 11: Have you had styes or chalazia before?

Answer Options:

- [ ] No, this is my first one
- [ ] Yes, once or twice before
- [ ] Yes, I get them frequently (3+ times per year)

Detailed Interpretation:

First time → Isolated incident

- May be due to temporary factors (rubbing eyes, makeup, stress)
- Less likely to have underlying chronic condition
- Good prognosis with appropriate treatment

Once or twice before → Consider risk factors

- May indicate intermittent eyelid hygiene issues
- Possible unrecognized blepharitis or MGD
- Review habits: eye makeup hygiene, contact lens care, hand washing

Frequent recurrence (3+ per year) → CHRONIC PROBLEM - NEEDS EVALUATION

Recurrent styes/chalazia indicate underlying issue:

Chronic blepharitis:

- Inflammation of eyelid margins
- Bacterial colonization (*Staphylococcus*)
- Dandruff-like flakes at lash base
- Red, irritated lid margins
- **Treatment**: Daily lid hygiene, warm compresses, antibiotic ointment, oral doxycycline for severe cases

Meibomian gland dysfunction (MGD):

- Oil glands produce thick, poor-quality secretions
- Glands become blocked repeatedly
- Associated with rosacea, dry eyes
- **Treatment**: Warm compresses with massage, omega-3 supplements, intense pulsed light (IPL) therapy, prescription eye drops

Rosacea:

- Skin condition with facial flushing, acne-like bumps
- Affects meibomian glands (ocular rosacea)
- **Treatment**: Oral doxycycline or azithromycin, topical metronidazole, trigger avoidance

Seborrheic dermatitis:

- Oily, scaly skin on scalp, face, eyelids
- Yeast (*Malassezia*) colonization
- **Treatment**: Anti-dandruff shampoo, tea tree oil lid scrubs

Diabetes:

- Impaired immune function increases infection risk
- Check blood sugar control
- May need more aggressive treatment

Action for frequent recurrence:

- See ophthalmologist or optometrist for comprehensive exam
- May need eyelid culture to identify bacteria
- Long-term prevention plan with daily lid hygiene
- Address underlying systemic conditions

Question 12: Do you have any of these conditions?

Answer Options (check all that apply):

- [ ] Acne or rosacea
- [ ] Dandruff or seborrheic dermatitis
- [ ] Diabetes
- [ ] Weakened immune system
- [ ] Chronic dry eyes
- [ ] None of the above

Detailed Interpretation:

Acne or rosacea → High risk for recurrent chalazia

- Rosacea causes ocular complications in 50% of patients
- Excess sebum production clogs meibomian glands
- May need oral antibiotics (doxycycline) for ocular rosacea
- Avoid triggers: spicy food, alcohol, hot drinks, extreme temperatures

Dandruff or seborrheic dermatitis → Associated with blepharitis

- Seborrheic blepharitis common with scalp seborrhea
- Requires regular lid hygiene with tea tree oil or diluted baby shampoo
- May benefit from anti-dandruff shampoo (selenium sulfide, ketoconazole)

Diabetes → Increased infection risk

- Poorly controlled blood sugar impairs immune function
- Staphylococcal infections more common
- May need more aggressive antibiotic treatment
- Ensure glucose control is optimized

Weakened immune system → NEEDS MEDICAL EVALUATION

Immunocompromised patients are at higher risk for:

- Severe infections spreading beyond eyelid
- Unusual organisms (fungi, atypical bacteria)
- Poor healing
- Complications

Causes of weakened immunity:

- HIV/AIDS
- Cancer chemotherapy
- Immunosuppressive medications (for organ transplant, autoimmune disease)
- Chronic steroid use

Action: See doctor promptly; may need cultures, systemic antibiotics, closer monitoring

Chronic dry eyes → Associated with MGD

- Dry eyes and chalazia often coexist
- Both related to meibomian gland dysfunction
- Treat both conditions together:
    - Warm compresses
    - Omega-3 fatty acids
    - Preservative-free artificial tears
    - Prescription eye drops (cyclosporine, lifitegrast)

None of above → Lower risk profile

- Likely isolated incident
- Good prognosis with standard treatment
- Still maintain good eyelid hygiene to prevent recurrence

Question 13: Do you wear contact lenses?

Answer Options:

- [ ] Yes
- [ ] No

Detailed Interpretation:

Yes → REMOVE LENSES IMMEDIATELY AND SEE DOCTOR

Contact lens wearers with eyelid infections have special considerations:

Higher risk of complications:

- Contact lenses can trap bacteria against eye
- Increased risk of bacterial keratitis (corneal infection)
- Difficult to distinguish between simple lid infection and sight-threatening corneal infection

Discontinue lens wear:

- Do not wear contacts until infection completely resolved
- Do not wear lenses on top of antibiotic ointment

- Discard daily lenses used during infection
- For monthly lenses: discard if infection suspected; otherwise, thoroughly disinfect
- Replace lens case

Possible giant papillary conjunctivitis (GPC):

- Chronic allergic reaction to contact lenses or deposits
- Can cause eyelid swelling and chalazion-like bumps
- Requires lens discontinuation and change in lens type/hygiene

Action:

1. Remove contact lenses immediately
2. See eye doctor same day or next day
3. Do NOT assume it's just a simple stye
4. Bring lenses and case to appointment for possible culture

No → Standard management based on other symptoms

Question 14: Do you use eye makeup?

Answer Options:

- [ ] Yes, regularly
- [ ] Yes, occasionally
- [ ] No

Detailed Interpretation:

Yes, regularly → Important hygiene considerations

Eye makeup and stye/chalazion connection:

How makeup contributes:

- Mascara, eyeliner, eyeshadow can block gland openings
- Old or contaminated makeup harbors bacteria
- Makeup remover residue can irritate glands
- Sleeping in makeup blocks pores and glands

Prevention measures:

- Remove ALL eye makeup before bed
- Use oil-free makeup remover or micellar water
- Replace mascara every 3 months
- Replace eyeliner every 6-12 months
- Never share eye makeup
- Avoid applying liner on inner waterline (blocks glands)
- Clean makeup brushes weekly

During active infection:

- **Discontinue all eye makeup until fully healed**
- Discard mascara and liquid eyeliner used during infection

- Eye shadow and powder products can be sanitized with 70% alcohol spray

Yes, occasionally → Lower risk but still relevant

- Follow same hygiene principles
- Replace products on schedule even if infrequently used (bacteria can grow in products)

No makeup → Eliminates one risk factor

- Focus on other causes if recurrent (blepharitis, rosacea, hygiene)

SECTION 4: Severity Assessment & Triage

Question 15: Do you have a fever?

Answer Options:

- [ ] Yes, temperature above 100.4°F (38°C)
- [ ] No

Detailed Interpretation:

Yes, fever → URGENT - SEE DOCTOR TODAY

Fever with eyelid infection suggests:

Preseptal cellulitis:

- Infection of eyelid tissues anterior to orbital septum
- Eyelid red, hot, swollen, tender
- Usually no vision changes or eye movement restriction
- Requires oral or IV antibiotics

Orbital cellulitis (more serious):

- Infection behind orbital septum, involving eye socket
- Fever, severe pain, proptosis (eye bulging forward)
- Restricted eye movements, double vision
- **Medical emergency** - can cause blindness or spread to brain
- Requires immediate hospitalization, IV antibiotics, possible surgery

Abscess formation:

- Large collection of pus requiring drainage
- Systemic signs of infection

Action:

- See ophthalmologist urgently or go to emergency department
- May need CT scan of orbits
- Will likely need systemic antibiotics (oral or IV)
- Children with preseptal/orbital cellulitis often require hospitalization

No fever → Localized infection

- Infection contained to eyelid
- Lower risk of serious complications
- Can often be managed with local treatment
- Still see doctor if other warning signs present

Question 16: Is the swelling spreading beyond the original lump?

Answer Options:

- [ ] Yes, entire eyelid is swollen
- [ ] Yes, swelling extending to cheek/eyebrow/other eye
- [ ] No, just localized to the bump area
- [ ] Getting worse rapidly (within hours)

Detailed Interpretation:

Entire eyelid swollen → NEEDS MEDICAL EVALUATION

- May indicate severe stye or abscess formation
- Possible preseptal cellulitis
- **Action**: See doctor same day; may need oral antibiotics

Swelling to cheek/eyebrow/other eye → EMERGENCY

- Preseptal or orbital cellulitis
- Risk of vision loss and serious complications
- **Action**: Emergency department immediately

Localized to bump → TYPICAL PRESENTATION

- Normal inflammatory response
- Continue warm compresses and monitor
- See doctor if not improving after 48 hours

Rapidly worsening (hours) → URGENT

- Unusual for simple stye/chalazion
- May indicate aggressive bacterial infection
- Possible necrotizing fasciitis (very rare but serious)
- **Action**: Emergency evaluation

SECTION 5: Treatment & Management

Question 17: Have you tried any treatments at home?

Answer Options:

- [ ] Warm compresses
- [ ] Over-the-counter antibiotic ointment
- [ ] Pain medication
- [ ] Ice/cold compresses
- [ ] Nothing yet
- [ ] Other home remedies (tea bags, aloe vera, etc.)

Detailed Interpretation:

Warm compresses → CORRECT FIRST-LINE TREATMENT

This is the most important home treatment:

How to do warm compresses properly:

1. Wash hands thoroughly
2. Use clean washcloth soaked in warm water (not hot - test on inner wrist)
3. Wring out excess water
4. Apply to closed eyelid for 10-15 minutes
5. Reheat cloth when it cools (every 2-3 minutes)
6. Repeat 4-6 times daily
7. After compress, gently massage the bump toward the eyelid margin to encourage drainage

Why warm compresses work:

- Heat melts thickened oils in blocked glands
- Increases blood flow to area, promoting healing
- Brings inflammatory cells to fight infection
- Encourages natural drainage of abscess

For chalazion: May take 4-6 weeks of consistent warm compresses to resolve

For stye: Usually improves within a week with regular compresses

Over-the-counter antibiotic ointment → LIMITED BENEFIT

- OTC options: bacitracin, polymyxin B/neomycin (e.g., Neosporin)
- May help for external stye if applied to lid margin
- **NOT effective** for chalazion (not infected)
- **NOT effective** for internal lesions (ointment doesn't penetrate to deep glands)
- Can cause allergic reaction (especially neomycin)

Better options: Prescription antibiotic ointment (erythromycin, bacitracin) if doctor recommends

Pain medication → APPROPRIATE FOR SYMPTOM RELIEF

- Ibuprofen (Advil, Motrin): Reduces pain and inflammation
- Acetaminophen (Tylenol): Reduces pain but not inflammation
- Follow package dosing instructions
- Take with food to prevent stomach upset

Ice/cold compresses → NOT RECOMMENDED

- Cold constricts blood vessels, reducing healing blood flow
- Does not melt oils in blocked glands
- May provide temporary numbing but doesn't treat underlying problem
- **Use warm, not cold**

Nothing yet → START WARM COMPRESSES NOW

- Most styes/chalazia respond to this alone
- The sooner you start, the faster resolution
- Set timer reminders to do compresses 4-6 times daily

Other home remedies → USE CAUTION

Tea bags:

- Some people use warm tea bags as compresses
- Rationale: Warmth plus possible anti-inflammatory compounds (tannins)
- **Risk**: Contamination, allergic reaction, eye irritation
- If using: Use fresh, clean tea bag; ensure not too hot; do not let tea drip into eye
- **Better**: Plain warm water compresses (same benefit, lower risk)

Aloe vera, honey, herbal pastes:

- **NOT RECOMMENDED** near the eye
- Risk of infection, severe allergic reaction, corneal damage
- No scientific evidence of benefit
- Can delay proper treatment

Do NOT use:

- Toothpaste
- Essential oils (especially tea tree oil undiluted)
- Castor oil
- Breast milk (popular myth, no evidence)
- Any substance not specifically designed for use near eyes

Question 18: Has the bump drained (pus came out)?

Answer Options:

- [ ] Yes, drained on its own
- [ ] Yes, I squeezed it (not recommended)
- [ ] No, still swollen and closed
- [ ] Partially drained but still swollen

Detailed Interpretation:

Drained on its own → EXCELLENT - NATURAL RESOLUTION

This is the desired outcome:

- Abscess ruptured through natural pressure
- Pus released, pressure relieved, pain decreases rapidly
- Healing accelerates after drainage

Post-drainage care:

1. Continue warm compresses 2-3 more days
2. Gently clean eyelid with diluted baby shampoo
3. Apply antibiotic ointment if prescribed
4. Do NOT squeeze to try to get more out

5. Watch for re-accumulation (if swelling returns, see doctor)

Expected timeline: Symptoms should improve 50% within 24 hours of drainage, resolve completely within 3-5 days

Squeezed it → RISKY - MONITOR CLOSELY

Why squeezing is dangerous:

- Can push infection deeper into tissues
- May cause scarring or damage to gland
- Risk of spreading infection to surrounding areas
- Can rupture gland inward, causing worse inflammation

What to do now:

1. Do NOT squeeze further
2. Apply warm compresses gently
3. Watch for worsening (increasing pain, redness, swelling)
4. If symptoms worsen, see doctor immediately (may have caused deeper infection)

If it seems better after squeezing: You got lucky, but don't do it again

Still swollen and closed → CONTINUE CONSERVATIVE TREATMENT

- Not all styes drain externally (some drain internally into conjunctiva)
- Chalazion often doesn't drain at all (not infected, just blocked)
- Continue warm compresses for at least 1-2 weeks
- If no improvement after 2 weeks, see doctor (may need incision & drainage or steroid injection)

Partially drained → GOOD PROGRESS

- Some drainage indicates abscess is opening
- Continue warm compresses to encourage complete drainage
- Keep area clean
- Should continue to improve over next few days

Question 19: Have you seen a doctor for this?

Answer Options:

- [ ] Yes, currently under doctor's care
- [ ] No, managing at home
- [ ] No, but planning to see doctor

Detailed Interpretation:

Yes, under doctor's care → FOLLOW PRESCRIBED TREATMENT

If doctor prescribed treatment, follow instructions exactly:

Antibiotic ointment (erythromycin, bacitracin):

- Apply thin ribbon to affected eyelid 2-4 times daily as directed

- Use clean hands or cotton swab
- Avoid touching tube tip to eye or lid
- Continue for full course even if improving

Oral antibiotics (doxycycline, azithromycin):

- For severe stye, preseptal cellulitis, or chronic blepharitis
- Take full course as prescribed
- Doxycycline: Take with food, avoid lying down for 30 minutes after, causes sun sensitivity
- Do not stop early even if symptoms improve

Steroid injection (triamcinolone):

- For persistent chalazion not responding to conservative treatment
- Injected directly into lesion
- May cause temporary bruising or skin lightening
- Repeat injection may be needed if initial injection only partially effective
- Success rate: 60-80% for chalazion

Incision & curettage (I&C) surgery:

- For large, persistent chalazion after 4-6 weeks of conservative treatment
- Done under local anesthesia in office or minor procedure room
- Incision made on inner eyelid surface, contents scooped out
- Minimal scarring (internal approach)
- Recovery: 1-2 weeks
- Post-op: Antibiotic ointment, warm compresses, avoid rubbing
- Possible complications: Bleeding, infection, recurrence (5-10%)

Follow-up:

- Return if symptoms worsen or don't improve as expected
- May need adjustment of treatment plan
- If recurrent, may need long-term prevention strategy

Managing at home → APPROPRIATE IF NO WARNING SIGNS

Home management is first-line for uncomplicated stye/chalazion:

Continue if:

- Symptoms mild to moderate
- No vision changes
- No fever or systemic symptoms
- Localized to eyelid
- Improving with warm compresses

See doctor if:

- Not improving after 48 hours (stye) or 2 weeks (chalazion)
- Getting worse
- Any warning signs develop

Planning to see doctor → GOOD DECISION IF:

- Symptoms severe or worsening
- Uncertain about diagnosis
- Previous treatment failures
- Recurrent problem
- Any warning signs present

Summary Decision Tree

SEEK IMMEDIATE EMERGENCY CARE (EMERGENCY DEPARTMENT) IF:

- **Vision changes**: Blurry vision, double vision, vision loss
- **Severe pain**: Eye pain (not just eyelid discomfort)
- **Photophobia**: Severe light sensitivity with pain
- **Fever >101°F (38.3°C)** with eyelid swelling
- **Spreading infection**: Swelling to cheek, eyebrow, or other eye
- **Eye movement problems**: Pain with eye movement, restricted movement
- **Eye bulging** (proptosis)
- **Rapidly worsening** symptoms (within hours)
- **Newborn/infant** with eyelid swelling (rule out orbital cellulitis, congenital obstruction)

SEE EYE DOCTOR WITHIN 24-48 HOURS IF:

- **Contact lens wearer** with any eyelid infection
- **Thick pus discharge** or very sticky eyelids
- **Persistent stye** not improving after 5-7 days of warm compresses
- **Chalazion** not improving after 2-3 weeks
- **Recurrent styes/chalazia** (3+ episodes per year)
- **Vision slightly blurry** (mild astigmatism from large chalazion)
- **Moderate photophobia** without severe pain
- **Immunocompromised** patient (diabetes, HIV, chemotherapy, immunosuppressive drugs)
- **Eyelid lump >1 month** (rule out tumor)
- **Atypical features**: Loss of eyelashes, irregular lump, bleeding, ulceration

---

HOME MANAGEMENT APPROPRIATE IF:

- **Typical stye**: Painful red bump on eyelid edge, <5 days duration
- **Typical chalazion**: Painless firm lump, <2 weeks duration
- **No warning signs**: Normal vision, no fever, no spreading, no severe pain
- **Improving** with warm compresses
- **Isolated incident** (not recurrent)
- **No high-risk factors**: Not contact lens wearer, not immunocompromised

Home care protocol:

1. Warm compresses 4-6 times daily, 10-15 minutes each
2. Gentle lid massage after compresses (toward lash line)
3. Eyelid hygiene: Clean with diluted baby shampoo or lid wipes

4. Pain relief: Ibuprofen or acetaminophen as needed
5. Discontinue eye makeup until healed
6. Avoid squeezing or popping
7. Do not wear contact lenses until resolved
8. Observe for warning signs

Expected timeline:

- **Stye**: Should improve 3-5 days, resolve 7-10 days
- **Chalazion**: May take 4-6 weeks; if no improvement by 6 weeks, see doctor

---

SCHEDULE ROUTINE OPHTHALMOLOGY APPOINTMENT IF:

- **Chronic problem**: Frequent styes/chalazia, chronic blepharitis
- **Associated conditions**: Rosacea, seborrheic dermatitis, dry eyes
- **Need long-term management plan**: Eyelid hygiene regimen, prescription medications
- **Previous treatment failures**: Multiple attempts at treatment without success
- **Cosmetic concern**: Residual lump after chalazion healing, scarring

---

Prevention Strategies

Daily Eyelid Hygiene (for those prone to recurrence)

Morning routine:

1. Warm compress 5-10 minutes
2. Gentle lid massage (circular motion from inner to outer lid)
3. Clean lid margins with lid scrub or diluted baby shampoo
4. Rinse with warm water
5. Pat dry with clean towel

Evening routine:

1. Remove all eye makeup with oil-free remover
2. Repeat warm compress and cleaning if prone to recurrence

Frequency: Daily for chronic blepharitis or MGD; 2-3 times/week for prevention

---

Makeup Hygiene

- Remove makeup nightly
- Replace mascara every 3 months
- Replace liquid eyeliner every 6 months
- Don't share makeup
- Apply eyeliner on outer lid only (not inner waterline)
- Use oil-free, hypoallergenic products if prone to problems
- Discard makeup during active infection

Contact Lens Care

- Wash hands before handling lenses
- Use proper cleaning and disinfection
- Replace lenses on schedule
- Never sleep in lenses (unless approved extended-wear)
- Never use tap water for cleaning
- Replace lens case every 3 months
- Give eyes regular "lens-free" days

General Prevention

- Don't rub eyes with unwashed hands
- Manage underlying conditions (diabetes, rosacea, dandruff)
- Take omega-3 supplements (may improve meibomian gland function)
- Stay hydrated
- Manage stress and get adequate sleep
- Avoid known allergens if allergic component
- Clean pillowcases regularly

For Chronic Recurrent Cases

Medical treatments:

- **Oral antibiotics**: Doxycycline 50 mg daily for 2-3 months (anti-inflammatory effect)
- **Topical antibiotics**: Erythromycin or azithromycin ointment to lid margin
- **Topical steroids**: For severe inflammation (short course only, under supervision)
- **Intense pulsed light (IPL) therapy**: For MGD associated with rosacea
- **LipiFlow or thermal pulsation**: Medical device treatment for MGD

Long-term maintenance:

- Daily warm compresses
- Weekly deep cleaning with lid scrubs
- Omega-3 fatty acid supplements (1000-2000 mg EPA+DHA daily)
- Manage systemic conditions (rosacea, diabetes, seborrhea)

Important Notes

This guide is for educational purposes only and should not replace professional medical evaluation. Styes and chalazia can usually be managed at home, but certain warning signs require prompt medical attention.

When in doubt, see an eye doctor. It's better to be evaluated and told you can manage at home than to delay care for a serious condition.

Do NOT self-diagnose serious eye conditions. Vision changes, severe pain, and spreading infection always require professional evaluation.

Children: Any eyelid infection in young children should be evaluated by a doctor, as they are at higher risk for complications and may have associated systemic illness.

Pregnant/breastfeeding women: Consult your healthcare provider before using any medications, including over-the-counter treatments. Warm compresses are safe.

Elderly patients: May have more severe infections due to age-related immune changes; lower threshold for seeking medical care.

Diabetic patients: Higher risk for severe infections; see doctor earlier rather than waiting to see if home treatment works.

Red flags requiring emergency care are emphasized throughout this guide -- **do not delay** seeking immediate medical attention if these are present.